

****DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 11

Age: 68 years old | Gender: Male

MRN: 000-TEST-11

Admission Date: [ADMISSION DATE] | Discharge Date: [DISCHARGE DATE]

Attending Physician: Dr. Michael R. Chen, MD

ADMISSION DIAGNOSIS & HOSPITAL COURSE

Patient 11 presented to Metro Medical Center via ED with acute exacerbation of COPD characterized by increased dyspnea, productive cough with yellow sputum, and decreased exercise tolerance × 3 days. Baseline GOLD Stage II COPD (FEV1/FVC ratio 52% on prior PFTs) with current presentation complicated by acute bronchospasm. Home O2 dependency at 2L NC; SpO2 on presentation was 87% RA, improved to 94-96% with supplemental oxygen. CXR revealed hyperinflation without acute infiltrate. ABG: pH 7.38, pCO2 48 mmHg, pO2 72 mmHg on 2L O2.

****Hospital Course:****

- Day 1-2: Initiated IV methylprednisolone 125 mg q.6h; aggressive bronchodilator therapy with albuterol/ipratropium nebs q.4h; levofloxacin 750 mg IV daily for presumed infectious trigger.
- Day 3-4: Transitioned to PO prednisone taper; continued nebulized therapy; O2 titrated to maintain SpO2 ≥90%. Patient demonstrated improved airflow on repeat spirometry (FEV1 improved 18% post-bronchodilator).
- Renal function stable; Cr 1.4 (consistent with CKD Stage 3a). BP well-controlled on home antihypertensives.
- Discharged on hospital day 5 in stable condition, ambulating independently, SpO2 94% on 2L NC at rest.

PERTINENT MEDICAL HISTORY

- COPD (moderate, GOLD Stage II)
- Hypertension (on lisinopril)
- Chronic Kidney Disease Stage 3a (baseline Cr 1.3–1.5)
- Former smoker (35 pack-year history; quit 8 years ago)

DISCHARGE MEDICATIONS

| Medication | Dose | Frequency | Duration |

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| Tiotropium (LAMA) | 18 mcg | Once daily inhaled | Ongoing |

| Formoterol/Budesonide (LABA/ICS) | 200/6 mcg | Twice daily inhaled | Ongoing |

| Prednisone (taper) | 40 mg → 20 mg → 10 mg → 5 mg | q.d. | Days 1, 3, 5, 7 post-discharge |

| Albuterol (rescue) | 90 mcg | q.4-6h PRN | Ongoing |

| Levofloxacin | 750 mg | q.d. PO | 5 days total (3 more days) |

| Lisinopril | 10 mg | q.d. | Ongoing |

| Omeprazole | 20 mg | q.d. | Ongoing (steroid prophylaxis) |

ACTIVITY & RESTRICTIONS

- Continue home O2 therapy; maintain 2L NC at rest and with activity; increase to 3L with exertion PRN to keep SpO2 ≥88%.
- Ambulation ad lib; no lifting >20 lbs × 4 weeks.

- Pulmonary rehabilitation strongly recommended; referral placed.

DIETARY